



CALIFORNIA LAW & ETHICS

PRACTICE PROTECTION TOOLKIT

Practical checklists, decision guides, and planning tools

California Law & Ethics for Mental Health Professionals:

Navigating the Evolving Clinical Landscape

RESOURCE TOOLKIT | VERSION 1.0 | JULY 2026

CTA STANDARD DISCLAIMER

This resource is intended for educational purposes only and does not replace clinical judgment, supervision, legal consultation, medical advice, or applicable laws and regulations.

Educating Clinicians. Strengthening Practice.

How to Use This Toolkit

This toolkit converts the course's central decision-making principles into reusable practice tools. Use the checklists to review existing procedures, prepare for difficult situations, support consultation, and document changes to practice systems.

CORE PRINCIPLE

Pause. Gather information. Identify the governing legal or ethical authority. Assess risk and clinical fit. Consult when appropriate. Take proportionate action. Document the reasoning.

Included Resources

Resource	Title	Primary Use
1	Regulatory Source and Competence Check	Identify authority, currentness, scope, competence, consultation, and documentation needs.
2	Informed Consent and Digital Communication Checklist	Review consent, fees, confidentiality, telehealth, messaging, social media, emergencies, and policy changes.
3	Confidentiality, Privilege, and Disclosure Decision Guide	Classify requests, verify authority, respond narrowly, and document decisions.
4	Working with Minors Decision Checklist	Separate consent authority, minor self-consent, parent involvement, custody, confidentiality, records, safety, and reporting.
5	Crisis Assessment and Duty-to-Protect Framework	Structure suicide- and violence-risk assessment, protective action, consultation, and documentation.
6	Documentation and Records-Request Checklist	Strengthen clinical notes, record access workflows, retention, security, and legal-request response.
7	Practice Continuity and Professional Will Planner	Plan for incapacity, death, practice closure, records, client notification, referrals, and business continuity.
8	Annual Law & Ethics Practice Audit	Review licensure, consent, records, technology, billing, advertising, crisis systems, and continuity.

Recommended Use

- ☐ Complete the relevant resource after finishing each course module.
- ☐ Use the toolkit during supervision, consultation, policy review, or annual practice planning.
- ☐ Adapt the prompts to your profession, setting, employer policies, and actual scope of practice.
- ☐ Verify current statutes, regulations, BBS guidance, professional ethical standards, and organizational procedures before relying on a legal detail.
- ☐ Do not place unnecessary client-identifying information in this educational workbook.

RESOURCE 1

Regulatory Source and Competence Check

Use before changing a legal-sensitive workflow, accepting a new population, adding a modality, or making a decision outside routine practice.

CLINICAL PEARL

Scope of practice tells you what the license generally allows. Scope of competence asks whether your education, training, experience, consultation, supervision, and judgment support providing the service safely.

A. Identify the Governing Source

Source Layer	Question	Source / Date Checked
Applicable law or regulation	What statute, regulation, or legal requirement may control?	_____
Licensing-board requirement	What current BBS source, form, FAQ, publication, or law-change notice applies?	_____
Professional ethical standard	What ethical principle or code provision informs the decision?	_____
Workplace or payer rule	What organizational, contractual, or payer requirement applies?	_____
Clinical standard of care	What accepted clinical practice, consultation, or specialty guidance is relevant?	_____

B. Competence Decision

☐ The service falls within the legal scope of my license or registration.
☐ I have relevant education and training for this population, diagnosis, risk level, or modality.
☐ I have supervised experience or other appropriate clinical experience.
☐ I understand the limits, contraindications, and foreseeable risks of the service.
☐ I have access to consultation, supervision, co-treatment, or specialty support.
☐ The setting, technology, documentation system, and emergency resources support safe delivery.
☐ The client has received accurate informed-consent information about the service.
☐ The case remains appropriate for my current competence as treatment evolves.

C. Choose the Proportionate Response

☐ Proceed within current competence.	☐ Proceed with consultation or supervision.
☐ Obtain additional training before proceeding.	☐ Use co-treatment or specialty collaboration.
☐ Limit the service temporarily.	☐ Refer or transfer to a clinician or level of care better matched to the client's needs.
☐ Reassess because the case has become more complex.	☐ Seek legal or risk-management consultation.

Decision and rationale:

Consultation obtained, recommendation, and effect on plan:

Follow-up date or trigger for reassessment:

RESOURCE 2

Informed Consent and Digital Communication Checklist

Use when creating or reviewing intake forms, consent conversations, telehealth procedures, communication policies, or social-media boundaries.

BIG IDEA

Informed consent is an ongoing process—not a one-time document. The client should have a realistic, understandable picture of the professional relationship.

A. Core Informed-Consent Elements

☐ Type and purpose of treatment or service.
☐ Clinician's role, license or registration status, qualifications, and supervision when applicable.
☐ Relevant risks, expected benefits, reasonable alternatives, and limits of the service.
☐ Client rights, responsibilities, voluntary participation, and opportunities to ask questions.
☐ Confidentiality protections and understandable explanation of the limits of confidentiality.
☐ Fees, billing, insurance, payment timing, cancellations, no-shows, late payment, and account concerns.
☐ Attendance, scheduling, emergency contact, after-hours, and crisis procedures.
☐ Records, documentation, access, storage, release, and privacy practices.
☐ Communication channels, typical response times, and what is not appropriate for urgent needs.
☐ How policy, fee, platform, clinician, treatment-unit, or service changes will be communicated and documented.

B. Telehealth-Specific Review

☐ Privacy and security risks in the clinician's and client's environments.
☐ Technology limitations and steps for connection failure.
☐ Client identity verification when appropriate.
☐ Client's current physical location and how location changes are handled.
☐ Emergency contact information and local emergency-resource planning.
☐ Approved platforms and communication channels.
☐ Expected response times and limits of monitoring.
☐ What the client should do during an urgent or emergency situation.
☐ Clinical appropriateness of telehealth and circumstances that may require in-person care or a different level of care.
☐ Backup plan for dropped calls, platform failures, loss of privacy, or sudden risk escalation.

C. Email, Text, Portal, and Messaging Policy

Policy Area	Practice Standard / Entry
Purpose of each channel	Scheduling only / forms / billing / clinical communication / other: _____
Typical response time	Business hours and realistic timeframe: _____
Emergency limitation	Messages are not monitored continuously; emergency instructions: _____
Security limits	Risks of shared devices, forwarding, lock-screen previews, interception, and storage explained.
Documentation	Define when clinically relevant messages are placed in the clinical record.
Boundaries	Clarify limits on therapy between sessions, message volume, and after-hours contact.
Staff consistency	Reception, billing, associates, supervisors, and contractors follow the same policy.

D. Social Media and Online Presence

☐ Policy states whether client friend/follow requests are accepted.
☐ Policy states whether direct messages are used for clinical communication.
☐ Plan addresses public comments or reviews without confirming a treatment relationship.
☐ Personal and professional accounts have been reviewed for foreseeable boundary and privacy concerns.
☐ Online searching for client information is guided by clinical purpose, consent considerations, and documentation.
☐ The written policy matches actual clinician and staff behavior.

Consent or policy changes needed:

How understanding will be checked and documented:

RESOURCE 3

Confidentiality, Privilege, and Disclosure Decision Guide

Use when a family member, payer, employer, attorney, court, Board, provider, or other third party requests information.

BIG IDEA

Confidentiality is the default. Disclosure is the exception. Every disclosure decision begins with authority.

A. Classify the Request

Request Type	Description	Primary Question
Ordinary third-party request	Family member, employer, payer, provider, spouse, or other person asks for information.	Confidentiality and authorization
Client-authorized disclosure	A valid authorization may permit a defined release.	Scope, recipient, purpose, expiration
Required disclosure	A defined law or order requires action when the threshold is met.	Trigger, timing, recipient, required content
Permitted disclosure	The law allows but does not necessarily require disclosure.	Necessity, client welfare, minimum information
Subpoena or legal demand	Legal process may involve confidentiality and privilege.	Do not ignore; do not automatically disclose
Court order	Review authenticity, scope, deadlines, privilege, and limits.	Respond narrowly; consult when needed
Not authorized	No valid authorization, law, exception, or legal process supports release.	Protect information; explain process as appropriate

B. Before You Disclose

☐ Identify exactly who is requesting information and verify identity.
☐ Identify what information, records, testimony, or confirmation is requested.
☐ Determine whether the issue involves confidentiality, privilege, or both.
☐ Identify who holds or may claim privilege and whether waiver or an exception is alleged.
☐ Confirm a valid authorization or other legal basis.
☐ Determine whether disclosure is required, permitted, or not authorized.
☐ Confirm that the facts satisfy the applicable legal threshold.
☐ Identify deadlines and whether immediate consultation is needed.
☐ Limit disclosure to what the authority and lawful purpose support.
☐ Use a secure method and verify the recipient.
☐ Communicate with the client when appropriate and legally permitted.

☐ Document the request, authority, assessment, consultation, decision, information disclosed or withheld, recipient, method, and date.

C. Subpoena Response Sequence

	Action	Practice Reminder
	Preserve	Date-stamp the request, preserve the record, and track the response deadline.
	Clarify	Determine what is requested, who issued it, and whether the patient or counsel notice.
	Protect	Do not automatically release records; evaluate confidentiality and psychotherapist privilege.
	Consult	Contact qualified counsel, malpractice carrier, supervisor, or risk-management representative as appropriate.
	Respond	Follow the legally appropriate process and respond only within the supported scope of practice.
	Document	Record the request, consultation, client communication, response, and materials disclosed or withheld.

Request received and deadline:

Authority considered and consultation obtained:

Decision, scope of disclosure, and rationale:

RESOURCE 4

Working with Minors Decision Checklist

Use before beginning treatment, responding to a parent request, managing custody conflict, evaluating self-consent, or addressing suspected abuse or neglect.

BIG IDEA

Consent authority, parent involvement, confidentiality, access to records, safety, and mandated reporting are related—but separate—questions.

A. Before Treatment Begins

☐ Identify the client and the requested treatment unit.
☐ Verify the identity and legal authority of the person requesting services.
☐ Determine whether legal custody is sole, joint, or otherwise limited.
☐ Review the actual current custody or guardianship order when relevant.
☐ Determine whether one or both parents must consent under the order and applicable law.
☐ Clarify the clinician's role and remain neutral in parent conflict.
☐ Identify whether a minor-consent law may apply.
☐ Clarify payment responsibility without confusing payment with authority or access rights.
☐ Discuss confidentiality, parent involvement, general progress updates, safety exceptions, and record-access limits.
☐ Document the legal basis and the facts relied upon before treatment begins.

B. Minor Self-Consent Review Under the Course Framework

on Area	Question	Finding
igibility	Is the minor at least 12 for the Family Code § 6924 pathway discussed in the course?	☐ Yes ☐ No ☐ Verify current law
e type	Is the request for qualifying outpatient mental health treatment or counseling?	☐ Yes ☐ No ☐ Unsure
ity	Can the minor explain the reason for treatment and participate intelligently in decisions?	☐ Supported ☐ Not supported
standing	Can the minor explain the nature, benefits, limitations, and confidentiality boundaries?	☐ Supported ☐ Not supported
t involvement	After discussion with the minor, would parent or guardian involvement be appropriate?	☐ Yes ☐ No ☐ Reassess
mentation	Were contact attempts, outcomes, or the reason involvement was inappropriate documented?	☐ Complete ☐ Needed

C. Confidentiality and Records

- ☐ Do not promise absolute secrecy to the minor.
- ☐ Do not promise the parent unrestricted access to everything discussed.
- ☐ Clarify what ordinarily remains private and what general information may be shared.
- ☐ Explain what must or may be disclosed for safety or legal reasons.
- ☐ Evaluate record-access rights separately from consent authority and parent involvement.
- ☐ Involve the minor in planned disclosures when possible and clinically appropriate.
- ☐ Revisit the plan as safety, treatment, custody, or family circumstances change.

D. Mandated-Reporting Decision

	Question	Entry
	What information created concern? Use objective, behaviorally specific details.	_____
hold	Do the facts create objectively reasonable suspicion under the applicable reporting law?	☐ Yes ☐ No ☐ Consult promptly
dual duty	Has each mandated reporter fulfilled the individual reporting obligation?	☐ Yes ☐ No
ent and timing	Was the report made to the legally designated agency within the required timeframe?	_____
en follow-up	Was required written follow-up completed and tracked?	☐ Yes ☐ N/A
al response	How will safety, the therapeutic relationship, and follow-up care be addressed?	_____

Consent/custody decision and legal basis:

Parent-involvement and confidentiality plan:

Reporting actions, consultation, and follow-up:

RESOURCE 5

Crisis Assessment and Duty-to-Protect Framework

Use to structure suicide-risk or violence-risk assessment, select a proportionate response, support consultation, and document the clinical reasoning.

CTA CRISIS FRAMEWORK

Pause. Assess. Clarify. Consult. Protect. Document.

A. Suicide-Risk Assessment

Assessment Area	Questions to Clarify	Findings
Statement / thoughts	Exact or close-to-exact language; passive, active, unclear, or changing.	_____
Frequency and intensity	Onset, duration, pattern, escalation, ability to dismiss or control.	_____
Intent	Desire or likelihood of acting; ambivalence; what the client believes may happen.	_____
Plan	Specificity, preparation, rehearsal, steps taken, or communications made.	_____
Means and access	Availability, immediacy, changes in access, and possible safety steps.	_____
Timeframe and immediacy	When the client might act and what increases near-term concern.	_____
History of behavior	Prior attempts, aborted attempts, preparation, recent self-harm, or escalating behavior.	_____
Contributing factors	Acute loss, hopelessness, agitation, intoxication, isolation, worsening symptoms, or other factors.	_____
Protective factors	Reasons for living, supports, future orientation, willingness to accept help, coping and safety strategies.	_____
Clinical decision	Risk formulation, level of care, intervention, follow-up, consultation, and rationale.	_____

B. Violence Risk and California Duty-to-Protect Questions

☐ What exactly was communicated, by whom, and in what context?
☐ Is the threat serious or potentially serious?
☐ Is the intended victim reasonably identifiable?
☐ What information supports or reduces credibility and immediacy?
☐ What access, preparation, history, intoxication, escalation, or contextual factors are present?
☐ Does the current California duty-to-protect statute appear to be triggered?
☐ What protective actions are required or clinically appropriate?
☐ Can protective action be taken while limiting unnecessary disclosure?
☐ What consultation is needed without delaying necessary action?

☐ How will the client's ongoing clinical needs and continuity of care be addressed?

C. Proportionate Crisis Response

☐ Continue assessment and increase monitoring.	☐ Collaborative safety planning and means-safety steps.
☐ Involve supports with appropriate authority or clinical justification.	☐ Increase session frequency or arrange rapid follow-up.
☐ Obtain supervisor, specialty, legal, or emergency consultation.	☐ Coordinate with another provider or crisis service.
☐ Arrange voluntary emergency evaluation or higher level of care.	☐ Initiate legally supported emergency or protective action.
☐ Warn or notify an identifiable victim and/or law enforcement when the applicable duty and legal framework support it.	☐ Document why a less restrictive or more restrictive response was selected.

D. Crisis Documentation

☐ Client's statements and relevant observable behavior.
☐ Questions asked and information gathered.
☐ Risk and protective factors considered.
☐ Structured tool or framework used, if any.
☐ Clinical formulation rather than a label alone.
☐ Consultation: purpose, information reviewed, recommendation, and effect on plan.
☐ Actions taken, persons contacted, information disclosed, and legal/clinical basis.
☐ Client response, collaboration, refusal, or barriers.
☐ Level-of-care decision and rationale.
☐ Safety plan, means-safety plan, follow-up, handoff, and contingency instructions.

Crisis formulation and rationale:

Actions, consultation, and follow-up plan:

RESOURCE 6

Documentation and Records-Request Checklist

Use to strengthen progress notes, record-request workflows, retention and destruction systems, and responses to attorneys, courts, clients, or the Board.

CLINICAL PEARL

Document the decision-making process—not only the final conclusion. A strong record shows the bridge from assessment to reasoning, decision, and follow-up.

A. Strong Clinical Note

☐	Relevant client report and observable information.
☐	Clinical assessment and meaningful changes since the prior contact.
☐	Interventions provided with enough specificity to show the clinical work.
☐	Client response, engagement, progress, disagreement, or barriers.
☐	Risk assessment and protective factors when clinically relevant.
☐	Clinical reasoning supporting diagnosis, level of care, treatment change, referral, or termination.
☐	Consultation purpose, recommendation, and effect on the plan.
☐	Follow-up plan, frequency, coordination, referrals, safety steps, and unresolved needs.
☐	Neutral, behaviorally specific language rather than unsupported character labels.
☐	Timely completion, accurate date, and proper late-entry or amendment process when needed.
☐	Templates and copied-forward fields reviewed for accuracy.
☐	Enough information for continuity—without turning the note into a transcript.

B. Record-Request Workflow

	Required Action	Done
	Date-stamp request; identify deadline; preserve relevant records.	☐
	Verify identity, role, and authority.	☐
	Clarify inspection, copies, summary, billing, dates, full record, testimony, subpoena, or court order.	☐
	Determine access rights, authorization, confidentiality, privilege, exceptions, and limits.	☐
	Use counsel, malpractice carrier, supervisor, or risk-management support when needed.	☐
	Respond within applicable deadlines and only within lawful scope.	☐
	Use secure delivery and verify recipient.	☐
	Record request, verification, analysis, response, date, method, and limitations.	☐

C. Retention, Security, and Destruction

- ☐ Written retention schedule reflects current profession-specific law and other applicable requirements.
- ☐ Adult and minor records are tracked under the retention framework taught in the course and rechecked against current law.
- ☐ Routine destruction stops when a complaint, subpoena, audit, litigation, or legal hold is anticipated.
- ☐ Paper and electronic records are stored with appropriate access controls.
- ☐ Backups are secure, tested, and recoverable.
- ☐ Former staff, contractors, and clinicians no longer have unauthorized access.
- ☐ Secure destruction protects confidentiality across paper, devices, media, cloud storage, and backups.
- ☐ Group-practice agreements define record custody and response responsibilities when a clinician leaves.
- ☐ Clients can find clear instructions for requesting records.

D. Board Complaint or Investigation

- ☐ Read the notice carefully and identify the request, case information, and deadline.
- ☐ Preserve all documentation and do not alter, backdate, rewrite, or disguise later additions.
- ☐ Notify the malpractice carrier promptly when coverage or assistance may apply.
- ☐ Seek qualified licensing counsel or risk-management consultation when appropriate.
- ☐ Respond professionally and within the required timeframe.
- ☐ Document internal actions while protecting applicable rights and privileges.
- ☐ Do not contact the complainant in a retaliatory, coercive, or confidentiality-breaching manner.

Documentation or records-system improvements needed:

Owner, deadline, and method for verifying completion:

RESOURCE 7

Practice Continuity and Professional Will Planner

Use to prepare for illness, incapacity, death, leave, abrupt practice closure, retirement, relocation, or another event that affects client care and records.

BIG IDEA

Continuity planning protects clients before an emergency occurs. The plan should make responsible action possible even when the clinician cannot personally carry it out.

A. Authorized Professional Representative

Planning Area	Entry
Primary representative / records custodian	Name: _____ License/role: _____
Contact information	Phone: _____ Email: _____
Alternate representative	Name / contact: _____
Authority and limits	Where the authority is documented and what the representative may do: _____
Access method	How the representative obtains necessary access without receiving unrelated access: _____

B. Client Care and Communication

☐ Method for identifying active clients and urgent/high-risk cases.
☐ Process for notifying clients of incapacity, death, closure, leave, or transition.
☐ Message templates that protect confidentiality and avoid unnecessary disclosure.
☐ Process for offering referrals, transfer, emergency instructions, and continuity options.
☐ Plan for handling clients who cannot be reached.
☐ Plan for pending appointments, waitlists, consultations, and group/family treatment.
☐ Process for documenting notification and transition efforts.

C. Records and Technology

☐ Location of paper and electronic records is documented securely.
☐ Instructions identify record custodian, retention schedule, legal holds, and destruction process.
☐ Secure access instructions exist for EHR, scheduling, billing, telehealth, phone, email, cloud storage, and backups.
☐ Passwords are not placed in a broadly accessible document; access uses a secure method.
☐ The plan addresses encryption, devices, keys, office access, and vendor contacts.
☐ Clients are told how to request records after closure or incapacity.

- ☐ Responsibility for responding to subpoenas, Board requests, and other legal process is assigned.

D. Business and Administrative Continuity

- ☐ License, professional association, malpractice carrier, and business contact information is current.
- ☐ Billing, refunds, outstanding balances, insurance claims, and superbills have an assigned process.
- ☐ Website, directory profiles, social media, voicemail, and automated scheduling can be updated or disabled.
- ☐ Lease, vendors, contractors, employees, and payroll obligations are addressed.
- ☐ Unused client payments, deposits, or records fees are handled ethically.
- ☐ Practice-sale, closure, retirement, or succession instructions are consistent with client rights and confidentiality.
- ☐ The plan is reviewed at least annually and after major practice changes.

E. Professional Will Summary

Element	Required Decision	Entry
Triggering event	How incapacity, death, or inability to practice will be confirmed.	_____
Decision authority	Who has authority to act and who provides legal guidance.	_____
Immediate priorities	Urgent clients, appointments, messages, records, and safety.	_____
Client notification	Method, wording, timing, and documentation.	_____
Records custody	Location, access, retention, requests, and destruction.	_____
Financial/admin	Billing, refunds, payroll, vendors, insurance, and business obligations.	_____
Public information	Website, directories, voicemail, email, and social media.	_____
Review schedule	Date last reviewed and next review date.	_____

Three continuity actions to complete in the next 30 days:

RESOURCE 8

Annual Law & Ethics Practice Audit

Complete before publication, at least annually, and whenever a material law, technology, service, or practice structure changes.

PRACTICE STANDARD

Compare written policies with actual behavior. Inconsistency between forms, workflows, staff practices, technology, records, billing, and public information is a recurring source of risk.

	Review Standard	Meets	Action
nd competence	License/registration status verified; renewal and CE tracking current; competence and consultation systems reviewed.	☐	☐
sources	Official BBS and other controlling sources bookmarked; law-change review process assigned.	☐	☐
nsent	Forms and conversations address current services, risks, fees, confidentiality, telehealth, records, emergencies, and changes.	☐	☐
munication	Actual email, text, portal, voicemail, and social-media practices match written policies.	☐	☐
	Location, identity, privacy, backup, emergency, jurisdiction, clinical-fit, and technology procedures reviewed.	☐	☐
lity and	Authorizations, minimum-necessary practices, privilege/legal-request workflow, and secure transmission reviewed.	☐	☐
custody	Consent authority, custody-order review, parent involvement, confidentiality, records, and minor-consent procedures reviewed.	☐	☐
eporting	Current thresholds, recipients, timing, written follow-up, individual duty, and internal escalation procedures reviewed.	☐	☐
nse	Suicide/violence assessment, duty-to-protect, consultation, local emergency resources, and documentation templates current.	☐	☐
tion	Notes are timely, clinically relevant, neutral, internally consistent, and show reasoning.	☐	☐
uests	Date-stamped intake, identity review, deadline tracking, consultation, secure delivery, and completion documentation tested.	☐	☐
nd security	Retention schedule, backups, access controls, legal holds, vendor exits, and secure destruction tested.	☐	☐
ees	Fees, cancellation practices, invoices, claims, superbills, refunds, and representations match actual practice.	☐	☐
	Name, license number/title, specialties, claims, fees, directories, websites, and profiles are accurate and current.	☐	☐
and continuity	Notice, referrals, transition, abandonment prevention, records, and follow-up procedures reviewed.	☐	☐
l will	Representative, records custody, client notification, technology access, and business continuity plan current.	☐	☐
plaint response	Notice routing, preservation, carrier notification, legal consultation, and response process established.	☐	☐

	Review Standard	Meets	Action
Contractors	Training, access, confidentiality, messaging, billing, and offboarding practices match policies.	☐	☐

Corrective Action Plan

Priority	Correction / Improvement	Responsibility
	_____	Owner: _____ Due: _____
	_____	Owner: _____ Due: _____
	_____	Owner: _____ Due: _____
	_____	Owner: _____ Due: _____
	_____	Owner: _____ Due: _____

How completion will be verified:

Personal Practice-Protection Action Plan

Choose concrete actions that can be observed and completed. A general intention such as “improve documentation” is less useful than a defined step such as “revise the crisis-note template and test it by August 15.”

	Specific Action	Deadline
	Action: _____	Due date: _____
	Action: _____	Due date: _____
	Action: _____	Due date: _____
	Action: _____	Due date: _____
	Action: _____	Due date: _____

Consultation, training, or legal resources I need:

The system I will use to track completion:

Date I will repeat the Annual Law & Ethics Practice Audit:

Course-Aligned Source List

- Clinical Training & Supervision Academy. California Law & Ethics for Mental Health Professionals: Navigating the Evolving Clinical Landscape. Revised course syllabus, July 2026.
- CTA Module 1: California Regulatory Frameworks & BBS Updates.
- CTA Module 2: Informed Consent & Digital Boundaries.
- CTA Module 3: Confidentiality, Privilege & Disclosure.
- CTA Module 4: Working with Minors: Consent, Crisis & Parents.
- CTA Module 5: Crisis Management, Tarasoff & Professional Liability.
- CTA Module 6: Documentation, Professional Practice & Licensure Protection.
- Primary sources identified within the course scripts include current California Board of Behavioral Sciences statutes, regulations, law changes, forms, publications, and disciplinary materials; California Family Code §§ 6924 and 3083; California Health and Safety Code §§ 123110 and 123115; California Penal Code § 11166; California Civil Code § 43.92; applicable profession-specific record-retention statutes; cited California court decisions; and the clinical suicide-risk guidance identified in Module 5.

LEGAL-UPDATE NOTICE

Requirements change. Verify current official sources and obtain qualified consultation before applying this toolkit to an actual case.